

إسم المريض ربيعاً : اميره ماهر حكيم حنا
المحافظة : الجيزة
رقم البطاقة : 28001182101968 رقم التليفون : 01278224267
العمر(تاريخ الميلاد) : 18-01-1980 المهنة : بدون عمل
العنوان : 32ش زكي السيسي
تاريخ تسجيل الإستمارة : 22-09-2026

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التعليق على التشخيص : -----

[illegible]

الاسم: د/محمد كيلاني
 الاسم: د/ندى حسن
 الاسم: د/نرمين رمضان
 الوظيفة: _____
 الوظيفة: _____
 الوظيفة: _____
 أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
 وتلتزم جهة العلاج بلائحة المجالس الطبية.
 كما تقر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د. | ----- / د. |

التشخيص النهائي :
 خطة العلاج :
 المستشفى : قسم :
 جهة العلاج : نفقات :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

--

Name: اميره ماهر حكيم حنا National ID: 28001182101968	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

أميرة
ماهر حكيم حنا

٢٢ ش. زكي السيدى - شعوبلا - الثلاثينى
بولاى الذكور - الجيزه

١٩٨٠ / ١٠ / ٢٨ ٢٨٠ ٠١ ١٨ ٢١ ٠١٩ ٦٨

IF8347290

صفحة في القرار → لكيمى
٦٢
ايعاى واسماعاى



٢٠١٩/١١ ٢٨٠٠١١٨٢١٠١٩٦٨

بدون عمل



أنشى
شهادة سعيد شهادى خذائى
البطاقة مارية حتى ٢٠٢٦/١١/١٢



04/23/2026

MDT FORM

Name:	حكيم حنا Maher امير	MRN:	9190
Age:	45 Y	ID:	28001182101968
Menopausal Sta		Phone:	01092582727
NRS SCORE:		Marital Status:	MARRIED
BMI:		Family History:	-VE
CO Morbidity:		Reason Of MDT:	POST OPERATIVE

Clinical Data:

BX: ILC AND MINOR IDC
ER: 8/8 PR: 6/8 HER2: +1 KI67: 25%

R

L

Sonomammography:Mets Workup:History:

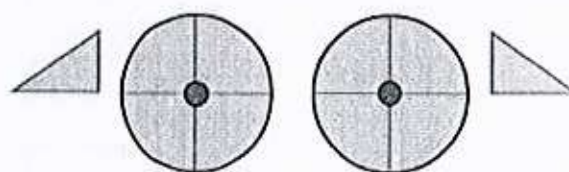
POST UPFRONT NIPPLE SPARING MASTECTOMY +ALND +SILICON
RECONSTRUCTION
-ILC+IDC
SKIN & NIPPLE& AREOLA : FREE
FREE MARGINS
L.NOD: 14/14
PT3,PTN3

Recommendations:

FOR ADJVENT

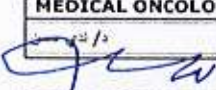
MDT Outcome & Plan

for adj Chemo after ECHO and Port-cath insertion: AC*4- TXT*4
PORT to WB and RNI (VMAT)
Endocrine ttt (Femara +Zoladex) for 5y and consider extended ttt
Zometa /6m with ET



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
					

Patient Name : الأستاذ / اميره ماهر حكيم
Patient No. : 28001182101968
Path No. : 410/26
Sex : Female
Age : 46 Y
Referring Doc :

Request No. : 13004997
Request Date : 11/02/2026
Verification Date : 18/02/2026
Financial Category : جوستاف روسي - مبادره
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Left breast (2 O'clock mass) mass (BIRADs 4c).
Nature of Specimen: Tru-cut biopsy, collected on 11/02/2026, at 01:30 pm.

Gross examination:

- Four grey white tissue cores, rangin in size between 0.4 cm and 1.5 cm long. Totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive lobular carcinoma with minor ductal component.
- * Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Lobular carcinoma in situ: Absent.
- * Angioinvasion: Absent.
- * Perineural invasion: Absent.
- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 30% of tumor cells (score 6/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).
- Ki67%: 25%.

Diagnosis:

Left breast mass, Tru-cut biopsy:

- Invasive lobular carcinoma with minor ductal component, Grade 2.

Verified by :

Dr. Duaa Mohamed

Printing Date : 24/02/2026 10:23:12

Page 1 of 3

Printing By : marwa mohamed youss



Patient Name : الأستاذ / اميره ماهر حكيم
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Financial Category : جوسفان رومي - مبادره
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 6/8).
- Her2: Low (score 1+).
- Ki67%: 25%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0 (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed

Printing Date : 24/02/2026 10:23:12

Page 2 of 3

Printing By : marwa mohamed youss



Patient's Name: Amira Maher Hakim
Date: 2026-03-16
Referred by: Prof.Dr. Triphasic CT
Hospital: Goustave Roussey International Egypt

REPORT

**MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER BOTH
ORAL & I.V. CONTRAST ADMINISTRATION TRIPHASIC: -**

Findings:

- The liver is enlarged in size with homogenous CT texture. Multiple bilobar focal lesions largest at IVb measures 11.4 mm showing faint peripheral enhancement in early phases with filling at delayed phase.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Clear scanned lung bases.

Opinion:

Hepatomegaly with small focal lesions too small to characterize yet benign pattern of enhancement eg hemangioma for better assessment by MRI abdomen dynamic.

Dr Mai Mohamed MD

Dr.Mai Sadiq

Dr.Mai Sadiq

Name : amira maher hakim
Branch : Portable
Ref.Physician Prof : ct cap+c

Date : 07-03-2026 02:17
Medical No. : GRI-2800118210968
Ins.Company :

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN
AND PELVIS REVEALED:**

I- CHEST:

- o Right middle lobar tiny pulmonary nodule measuring about 2mm.
- o Clear rest of both lungs showing no sizable pulmonary nodules, consolidation or cavitation.
- o No pathologically enlarged hilar or mediastinal lymph nodes.
- o No plural or pericardial collection.
- o CT signs of cardiomegaly

II- ABDOMEN AND PELVIS:

- o Enlarged liver showing few hypodense hepatic focal lesions, largest one seen at segment IVb measuring about 9x12mm. nodilated biliary radicles.
- o Normal C.T appearance of the spleen, pancreas, both kidneys, suprarenal glands, aorta and IVC.
- o The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- o Normal appearance of the pelvic organs with no pelvic masses or collections.
- o IUD is noted
- o No pathologically enlarged abdominal or pelvic lymphadenopathy.
- o No ascites.

Opinion:

- ❖ Fatty hepatomegaly showing few hepatic focal lesion for better assessment by triphasic CT
- ❖ Right middle lobar tiny pulmonary nodule
- ❖ Cardiomegaly for echo correlation

Dr. Nourhan Mohamed,MD

08/03/2026 05:28 AM



خطابات أورام

NAME: Amira Maher Hakim	AGE: 46-year-old female
DATE: 9-3-2026	STUDY ID: 462944

DYNAMIC CONTRAST-ENHANCED BREAST MRI (DCE-BMRI)

FILMS COMPARED:

No Prior MRI Images available for comparison.

TECHNIQUE:

Localizer, Axial T1& T2, T1 pre +C non-FS, T2 FS/STIR, T1pre +C FS, Dynamic T1 post +C FS, Subtraction Images, DWI/ADC.

HISTORY AND INDICATION:

46-year-old female known case of left breast Cx for disease extension assessment.

FINDINGS:

- Breast Parenchymal Density: ACR MRI-C, heterogeneous fibroglandular tissue.
- Background Parenchymal Enhancement: is mild and asymmetrical.

RIGHT BREAST:

- Dilated retro areolar ducts = 4 mm
- No suspicious masses, enhancements.
- No enlarged axillary lymph nodes

LEFT BREAST:

- UOQ abnormal suspicious regional area of Non-Mass Like Enhancement (NMLE) with partial extension to the UIQ and LOQ which elicit low signal on both T1WI and T2WI and heterogeneous enhancement on post contrast images measures in its maximum dimensions about (9.3x4x6 cm)=(APXTSXCC).
- No enlarged axillary lymph nodes



خطابات أورام

N.B.: hepatic focal lesion seen at segment 8 measures about (1.6x1.5 cm) with marginal faint enhancement for dynamic liver MRI

IMPRESSION:

---RIGHT BREAST: Dilated retro-areolar ducts— Duct ectasia (BI-RADS 2)

---LEFT BREAST: Pathologically proven case of breast Cx showing UOQ abnormal regional area of Non-Mass Like Enhancement (NMLE) (that involved all UOQ quadrant with partial extension to the UIQ and LOQ) (BI-RADS 6)

N.B: Hepatic Focal Lesion As Described

RECOMMENDATION:

--- Clinical Management

BEST REGARDS

Dr. Heba Nabil, MD
RADIOLOGY & WOMEN'S IMAGING CONSULTANT

Dr. Rayman Mousad
SENIOR RADIOLOGY R.



Presidential Program for Women's health

Name: Amira Maher Hakim Hana

Date: 9-2-2026

Bilateral Full field Digital Mammography

Clinical summary: 45year old female with left breast palpable lump

Technique

Mediolateral oblique and CC views

High resolution ultrasound

Detailed Findings

Right breast

- Dense Heterogenous parenchyma breast (ACR c).may obscuring small lesion
- No asymmetric densities, distortion or suspicious clusters of microcalcifications.
- **Ultrasound** demonstrated retroareolar anechoic dilated ducts 30 mm
- No masses or cysts.
- Normal skin/nipple/areolar complex.
- Preserved axillary LNs.

Left breast

- Dense Heterogenous parenchyma breast (ACR c).may obscuring small lesion
- UOQ asymmetry with distortions
- **Ultrasound**
- Corresponding to mammo finding, at 2 o'clock zone c shows irregular spiculated Heterogenous solid lesion, harboring calcifications, the lesions measuring 27xx28mm with penetrating blood vessel by color Doppler, it is away from skin 6mm and nipple by 10cm
- small anechoic cyst seen anterior to lesion measuring 12m
- Dilated retroareolar ducts 30mm with
- Normal skin/nipple/areolar complex.
- Enlarged multiple lymph nodes, largest 13x7mm and focal cortical thickening 3 mm (Indeterminate).

Conclusion:

-Right breast: (BIRADS 2) ductectesia

-Left breast: (BIRADS 4c), malignant looking soil lesion for biopsy

Best regards,

Dr. Marwa M.Hassan

Patient Name : اميره ماهر حكيم حنا
Patient Info. : Female 45Y
Department : Out Patient
Profile NO. : 740-2026

Patient Id : 16327
VisitId : 180528
Collection Date : 07/03/2026 12:14:24
Reporting Date : 07/03/2026 12:51:06

Clinical Chemistry Report

Test	Result	Unit	Reference Range
<u>Kidney Function</u>			
Blood Urea	: 23.9	mg/dL	15.6 - 45
Serum Creatinine	: 0.71	mg/dL	0.5 - 0.9

Signature

Dr. Radwa Mohammed



نتائج أدق... صحة أفضل

معمل

صحتك
للتحاليل الطبية وأبحاث الدم

Name	الأستاذة / اميرة ماهر حكيم	Patient ID	3394
Sex / Age	Female - 45 Years	Visit Date	2026-03-08 9:57:49 PM
Referred By	Herself	Print Date	2026-03-08 9:59:04 PM



103550

Kidney Functions

Test Name	Result	Unit	Reference range
■ Creatinine	1.05	mg/dL	(0.6 - 1.1)

Doctor's signature

معمل صحتك
للتحاليل الطبية
وأبحاث الدم

78 شارع العشرين فيصل - تقاطع ترعة عبد العال 1 أمام كشري رمضان

☎ 01557793617 ☎ 01156316961

دوسلفا
روستاني
٨ - ٢٦



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الجمهورية
مصر

08/26/2026

MDT FORM

Name: حكم طاهر ابو	MRN: 9190
Age: 47 Y	ID: 28001182101968
Menopausal Sta: premenop	Phone: 01092582727
NRS SCORE:	Marital Status:
BMI:	Family History:
Gender:	Reason Of MDT:
CO Morbidity:	pt is on adjuvant chemo with newly discovered l5 vertebral lesion likely metast
Surgical History: * left mastectomy and reconstruction	ECOG Performance Status ☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4

Clinical Data:

R

L

BX:

ER:

PR:

HER2:

KI67:

History:

premielft breast cancer undergone on 5/4/2026 left ssm and reconstruction
 pT3 n3 14/14 lIc 2 er8/8 pr6/8 her2-ve ki 25%
 cts rt middle lobe tiny pulmonary nodule few hfi triphasaic ct small hfi
 too small to be categorized 6/2026
 pt rec 4 ac and 4 weeks taxol last 25/8/2026
 while on adjuvant cxhermotherapy
 bone scan on30/7/2026 :
 focal uptake of LV5 vfor radiological corr
 mri abdomen 10/8/2026 few bilobar hfi likely typical and atypical
 hemangioma LV5 sclerotic lesion likely metastatic
 mri LS VERT 17/8/2026 l5 vertebral body infiltrative lesion likely
 metastatic for mdt (radiological review and further plan of adjuvant it)

Sonomammography:

Clinical Staging:

Biomarker/Molecular:

Pathological Staging:

Mets Workup:

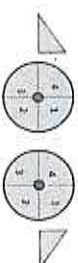
Recommendations:

MDT Outcome & Plan

MRI METS IN L5 FOR BASE LINE CT CAP + CEA CA 15.3 + MRI WHOLE
 SPINE
 PALLATIVE RTH IF PAIN
 ZOLDEX FEMARA
 ZOMETA / 3 MONTHS

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
3 محمد كلالى	3 محمد شادى		



T N M

Right Side
 Q1 Q2 Q3 Q4
 Left Side
 Q1 Q2 Q3 Q4

دوسناني
روستاني
٨-٢٦



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

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مصر

08/26/2026

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Sonomammography:

Clinical Staging:

Biomarker/Molecular:

Pathological Staging:

Mets Workup:

Recommendations:

MDT Outcome & Plan

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 ZOMETA / 3 MONTHS

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
3 محمد كرامي	3 محمد شادي		

T N M

Right Side Left Side
 Q1 Q2 Q3 Q4 Q1 Q2 Q3 Q4

دستار
روستار
مستشفى
مستشفى



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

مستشفى
مستشفى

08/26/2026

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Sonomammography:

Clinical Staging:

Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4
Right Side				Left Side			

T N M



Biomarker/Molecular:

Pathological Staging:

Mets Workup:

Recommendations:

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ZOMETA / 3 MONTHS

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
------------------	---------	-----------	-----------

3 محمد كمال
3 محمد كمال
3 محمد كمال

دوسلاف
روستافي
٨-٢٦



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

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08/26/2026

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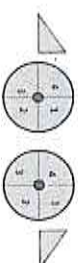
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MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
3 محمد كلالى	3 محمد شادى		

T N M



Q1 Q2 Q3 Q4 Right Side Q1 Q2 Q3 Q4 Left Side

Medical No. : 7979
Patient's Name: Amira Maher Hakim Hana
Date: 2026-09-14
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



**MULTISLICE CT EXAMINATION OF THE NECK, CHEST, ABDOMEN AND PELVIS
WITH CONTRAST ADMINISTRATION REVEALED:-**

- Normal appearance of the naso-, oro- and hypopharynx with preserved parapharyngeal fat planes.
- Intact laryngeal skeleton with no abnormally-enhancing glottic, supra or infra glottic masses.
- Normal course and calibre of the carotid and jugular vessels.
- Normal CT appearance of the submandibular and parotid salivary glands showing homogenous enhancement. No focal solid or cystic lesions.
- Normal thyroid gland.
- **Few subcentimetric bilateral upper deep cervical and submandibular LNs are noted; likely to be non-specific.**
- Bilateral pathologically-enlarged supra-clavicular lymph nodes are seen ,larger on the left side and globally measuring about 36x34mm in dimensions.
- No other abnormally-enhancing pathologically enlarged cervical lymph nodes.
- A left breast silicone implant with some irregularities seen at its inner aspect ,few thickened internal trabeculations and the overlying skin is thickened ,reaching to about 11mm.
- Normal right breast .
- **Benign-looking right axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 20 mm.**
- Hyperinflated both lungs.
- A thin linear atelectatic band is seen in the anterior segment of left lower pulmonary lobe.
- Vanished the right nodule previously-noted in the middle lobe.
- No sizable abnormally-enhancing pulmonary nodules ,patchy areas of consolidations or foci of pulmonary calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- **Border line size of the heart with normal CT of its big vessels.**
- Newly-developed foci of osteosclerotic are seen scattering in the scanned skeleton with the largest one seen in LV5.
- Mildly increased sized liver,measuring about 17 cm, showing a homogenous reduced parenchymal attenuation and a homogenous enhancement pattern.
- No appreciable changes seen as regards to the Ct criteria of the lesion previously-noted in segment 1Vb.
- No other newly-developed sizable abnormally-enhancing hepatic focal lesions or dilated biliary radicles.

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- Patent PV.
- Mildly increased sized spleen, measuring about 14.2 cm. No focal lesions seen inside.
- Relatively thickened both adrenal glands.
- Normal CT appearance of the pancreas, functioning both kidneys, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones, solid masses, wall thickening or diverticulosis seen.
- An IUCD is seen in the uterine cavity.
- Bilateral tiny ovarian cyst is seen and each measures about 22x22mm and 26x21mm in dimensions respectively, likely to be follicles.
- The rest of the pelvic structures are unremarkable.

IMPRESSIONS:

A status follow up ,and comparing with the previous study done at 07/03/2026, the current examinations shows:

- A left breast silicone implant with some irregularities at its inner aspect ,few thickened internal trabeculations and the overlying skin is thickened ,denoting a partial disruption herein. For clinical check up.
- Newly-developed bilateral pathologically-enlarged supra-clavicular lymph nodes, foci of osteosclerosis scattering in the scanned skeleton, and relatively thickened both adrenal glands, likely to be metastases.
- A thin linear atelectatic band in the anterior segment of left lower pulmonary lobe.
- Vanished the pulmonary nodule previously-noted in the middle lobe, denoting a complete resolution.
- Mild hepatomegaly with fatty infiltration and no appreciable changes seen as regards to the CT criteria of the lesion previously-noted in segment 1Vb, denoting a stable course.
- Mild splenomegaly.

Prof/Hanan El-Ahwal.

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Dr.Hanan Elahwel